Few labels carry as much unfair stigma as this one, including from professionals who should know better. Whether this belongs under the neurodivergent umbrella is genuinely debated. Many people who have it identify as neurodivergent; some prefer not to. Both are reasonable, and nobody gets to decide it for you. Many people prefer 'emotional intensity' or 'emotionally unstable personality disorder', and some reject the label entirely.

In plain words

BPD involves intense emotions that arrive fast, run hot, and take longer than usual to come down; a fear of abandonment; and an unstable sense of self. It often follows childhood trauma, though not in every case, and it overlaps heavily with autism and ADHD — many women in particular are given this label when autism would fit better.

What it can feel like from the inside

- Feeling everything at maximum volume, then being told you're overreacting.
- A small change in someone's tone reading as certain rejection.
- Loving people intensely and being terrified they'll leave.
- Not being sure who you are when you're on your own.
- Emotions passing eventually — and the shame afterwards lasting far longer.

What people often get wrong

THE ASSUMPTION	WHAT'S ACTUALLY GOING ON
"They're manipulative."	That word has done enormous damage. It's distress, communicated badly because nothing else worked.
"It's untreatable."	It has one of the better recovery rates of any diagnosis, especially with DBT.
"They're attention seeking."	Feeling a need for deep connection, showing as distress when nothing else has worked.
"It's a personality flaw."	In some cases, though not all, an adaptation to an environment that wasn't safe.
"It is said that women get this, men get PTSD."	A criticism raised from within the field itself, and one reason the label is so contested.

What often comes with it

NOT COMPENSATION PRIZES — GENUINE, USEFUL THINGS

Emotional depth and honesty most people never reach.

Extraordinary empathy — they feel it with you.

Loyalty and passion in relationships.

Perceptiveness about what people are really feeling.

Real capacity for change — this is a diagnosis people genuinely recover from.

What actually helps

- ☐ Be consistent — say what you'll do, then do it
- ☐ Validate the feeling before the facts
- ☐ Support DBT or similar therapy
- ☐ Don't withdraw suddenly without explanation
- ☐ Keep boundaries kind and predictable
- ☐ Never use 'manipulative'

If someone tells you this is them

HELPFUL

“That sounds really painful.”

“I'm not going anywhere.”

“I'll message you at six, and I will.”

LESS HELPFUL

“You're being manipulative.”

“Calm down.”

“Here we go again.”

Intensity is not a character defect. And this is one people do recover from.

If you are struggling right now, you don't have to wait until it gets worse. In the UK you can call Samaritans free on 116 123, any time, or contact your GP. Elsewhere, your local crisis line or doctor is the place to start.

A friendly explainer written for the community — not a diagnostic tool and not medical advice. If you want an assessment, your GP is the place to start. Nobody experiences this identically, so take what fits and leave the rest.

Free to share — print it, forward it, hand it to anyone who needs it. More free guides, one per neurotype, at froglogic.co.uk